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Data & Finance Analytics · London, UK

CONSULTING CASE STUDY · PRACTERA MICRO INTERNSHIP

Strategic funding opportunities for aidKRIYA.

A weighted-scoring approach to India's health-funding landscape: 22 funders screened against five criteria, a ten-organisation shortlist, and a recommendation that the client **stop leading with a venture-capital pitch**.

Weighted scoring model

Investor & grant research

Data reconciliation

Excel

Client reporting

22

funders screened and scored

10

prioritised shortlist

5

weighted criteria

8

clear the 80-point threshold

CLIENT

Gita & Susil Kumar Das Foundation (GSKDF) — aidKRIYA

PROGRAMME

King's College London × Practera Consulting

ROLE

Student Consultant — Research Lead, Team FinEdge

PERIOD

Jun – Jul 2026

Find money that **actually** fits.

aidKRIYA is a technology-enabled social venture supporting people living with rheumatoid arthritis, inflammatory bowel disease and other chronic autoimmune conditions. The client asked for venture-oriented investors. The research found the honest answer was more complicated — and more useful.

22

Organisations screened and scored

10

Prioritised shortlist delivered

5

Weighted evaluation criteria

4

Distinct funding routes mapped

The client

GSKDF is an Indian Section 8 not-for-profit company. Its flagship initiative, aidKRIYA, combines four assets: **Walker App** (digital patient support), **aidABLE** (assistive mobility), **Purplepages** (resource navigation) and **AIDothon** (a student health-innovation hackathon, now in its fourth edition with IIT Ropar). Together they reach patients, caregivers and students across 22 Indian states.

The objectives

- Identify suitable venture-oriented investors for aidKRIYA.
- Assess strategic alignment against each funder's stated investment focus.
- Recommend the top 10 opportunities with the strongest potential for long-term collaboration.
- Provide practical guidance on engagement, positioning and sequencing.

Method

Secondary research across funder websites, annual reports, portfolios, Crunchbase, Tracxn, LinkedIn and healthcare investment publications, cross-checked against the client briefing and aidKRIYA's own publications. Every candidate was then scored on the same five-criterion framework so that comparisons were like-for-like rather than narrative.

MY ROLE

Research Lead

Central point of contact for data requests across an eight-person team — gathering, validating and reconciling funder data from sources that frequently disagreed with each other.

Owned the alternative-funder workstream (corporate CSR, corporate foundations and philanthropic trusts sitting outside the healthcare-VC brief) and the deep-dive analyses on Azim Premji Foundation, BioAngels/IAN-BIRAC and HealthQuad.

Findings were consolidated into a single scored master sheet and presented to the client's Managing Director.

Team FinEdge

Sze Synn Lim · Palak Sehgal · Yufei Fan · Yi-Yu Chang · Yingying Wang · Gunjan Jain · Dhruv Kamdar · Varoon Rajesh

From longlist to recommendation

Screening funnel — each stage applied the same five-criterion test

Longlist screened

40

candidate organisations gathered

Scored on 5 criteria

22

carried into the weighted model

Shortlisted

10

recommended to the client

High priority

8

cleared the 80-point threshold

One scoring model, applied to everyone.

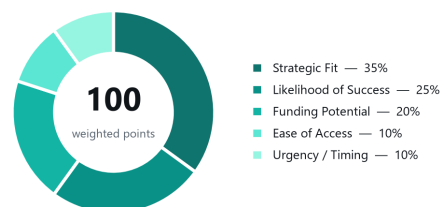
The client's constraint was realism, not ambition: they wanted funders they could plausibly win, not the funders with the deepest pockets. That judgement is encoded directly in the weights.

CRITERION	WEIGHT	ASSESSMENT FOCUS
Strategic Fit	35%	Alignment between the funder's mission and investment thesis and aidKRIYA's healthcare focus and long-term objectives.
Likelihood of Success	25%	Evidence of previous investment in or support for organisations with similar missions, sectors or technologies.
Funding Potential	20%	Expected capacity to provide meaningful financial support given investment size, portfolio and scale.
Ease of Access	10%	Accessibility of decision-makers, clarity of the application process and feasibility of initiating engagement.
Urgency / Timing	10%	Current investment activity, open funding windows and likelihood of engagement in the near term.

Table 1 — Evaluation framework. Each criterion is scored 1–5 against a published rubric; the weighted total is scaled to 100.

Weight distribution

Fit and precedent carry 60% of the model



Worked example — Social Alpha

Strategic Fit	5 × 35%	1.75
Likelihood of Success	5 × 25%	1.25
Funding Potential	4 × 20%	0.80
Ease of Access	4 × 10%	0.40
Urgency / Timing	5 × 10%	0.50
Weighted total	4.70 × 20	94

Applied identically to all 22 organisations. Every score in this report reproduces from its five inputs.

WHY FIT OUTRANKS MONEY

Strategic Fit was weighted highest because the client explicitly asked for realistic, long-term funding relationships rather than the largest available cheque. Likelihood of Success came second to reflect the goal of *sustainable* support. Ease of Access and Urgency were deliberately held to 10% each — they affect how quickly a conversation starts, not whether it should happen at all.

Scoring rubric — what each point means

SCORE	STRATEGIC FIT (35%)	LIKELIHOOD OF SUCCESS (25%)
5	Mission and investment focus highly aligned with aidKRIYA	Strong evidence of supporting very similar organisations
4	Strong alignment with one or more of aidKRIYA's focus areas	Frequently supports organisations in related sectors
3	General alignment with healthcare, technology or social impact	Some evidence of relevant interest, but not a clear focus
2	Limited relevance to aidKRIYA's work	Limited evidence of supporting similar initiatives
1	Little to no relevance	No clear evidence of interest or support

Table 2 — Published rubric for the two highest-weighted criteria. Funding Potential, Ease of Access and Urgency / Timing follow the same 1–5 structure, from "unlikely / invitation only / no current opportunity" at 1 to "substantial / easily accessible / active funding window" at 5.

What Indian health funders are **actually** buying.

Four trends shape which door aidKRIYA should knock on — and why grant and accelerator capital outranked venture capital in the final recommendation.

01

Health is the fastest-growing giving segment

Public-sector social spend has grown at roughly **13% annually over five years**, driven by the 2030 SDG agenda — expanded insurance access, remote consultation and community health melas. A favourable backdrop for technology-enabled, community-based delivery.

Source: Bain & Company, India Philanthropy Report 2025

02

Digital health is now a distinct investment focus

Strategic buyers and private equity continue to target technology-driven healthcare assets. Against demographic shift, rising chronic disease and tighter budgets, innovation-led digital health is being treated as **growth capital, not cost**.

Source: EY healthcare M&A and investment outlook

03

Preventive care is the highest-frequency "bold bet"

India's chronic disease burden is rising, emerging earlier and escaping routine checks. With **~80% of conditions addressable at primary and preventive level**, a Bridgespan × NATHEALTH survey found strengthening preventive care was the single most-cited priority.

Source: Bridgespan & NATHEALTH survey

04

Funders want seed capital, not perpetual subsidy

Capital is increasingly expected to build **sustainable revenue streams** rather than fund one-off initiatives. This outcome-orientation now runs through CSR, venture philanthropy, impact investors and family offices alike.

Source: CSRBOX, Indian CSR Outlook Report 2025

POSITIONING INSIGHT

Lead as a health-impact platform, not a disease-specific charity.

India's health funding is shifting towards scalable, technology-enabled and preventive models, and aidKRIYA already holds the assets that fit. The problem was never the substance — it was the framing. Presented as a niche autoimmune charity, aidKRIYA reads as small; presented as a chronic-condition health-impact platform spanning mobility, digital support, navigation and innovation, it maps onto what these funders say they are looking for. That reframing is the single highest-leverage change available before any outreach begins — and it costs nothing.

Four routes, sequenced

ROUTE	PRIORITY	ORGANISATIONS IN THE TOP 10
Accelerators & incubators	FIRST	Social Alpha · Villgro · Attvaran Accelerator · C-CAMP
Impact & venture philanthropy	FIRST	ACT For Health / ACT Grants
Family offices & foundations	FIRST	Azim Premji Foundation
Healthcare / MedTech VC	LATER	BioAngels / IAN-BIRAC · HealthQuad / HealthKois · Indian Healthcare Angels
Strategic healthcare partners	PARALLEL	Apollo Foundation / Apollo Hospitals ecosystem

Table 3 — Investor routes. VC routes activate only once user feedback, a signed partner and a clear revenue structure exist.
DK. Strategic Funding Opportunities for aidKRIYA · Practera Micro Internship

Every funder, on one scale.

Scoring the full set on identical criteria produced a clear separation. Eight organisations cleared 80. Notably, the three best-known generalist venture funds finished last.

Weighted funder scores — full screened set

22 organisations · scored out of 100 · colour denotes assigned priority band

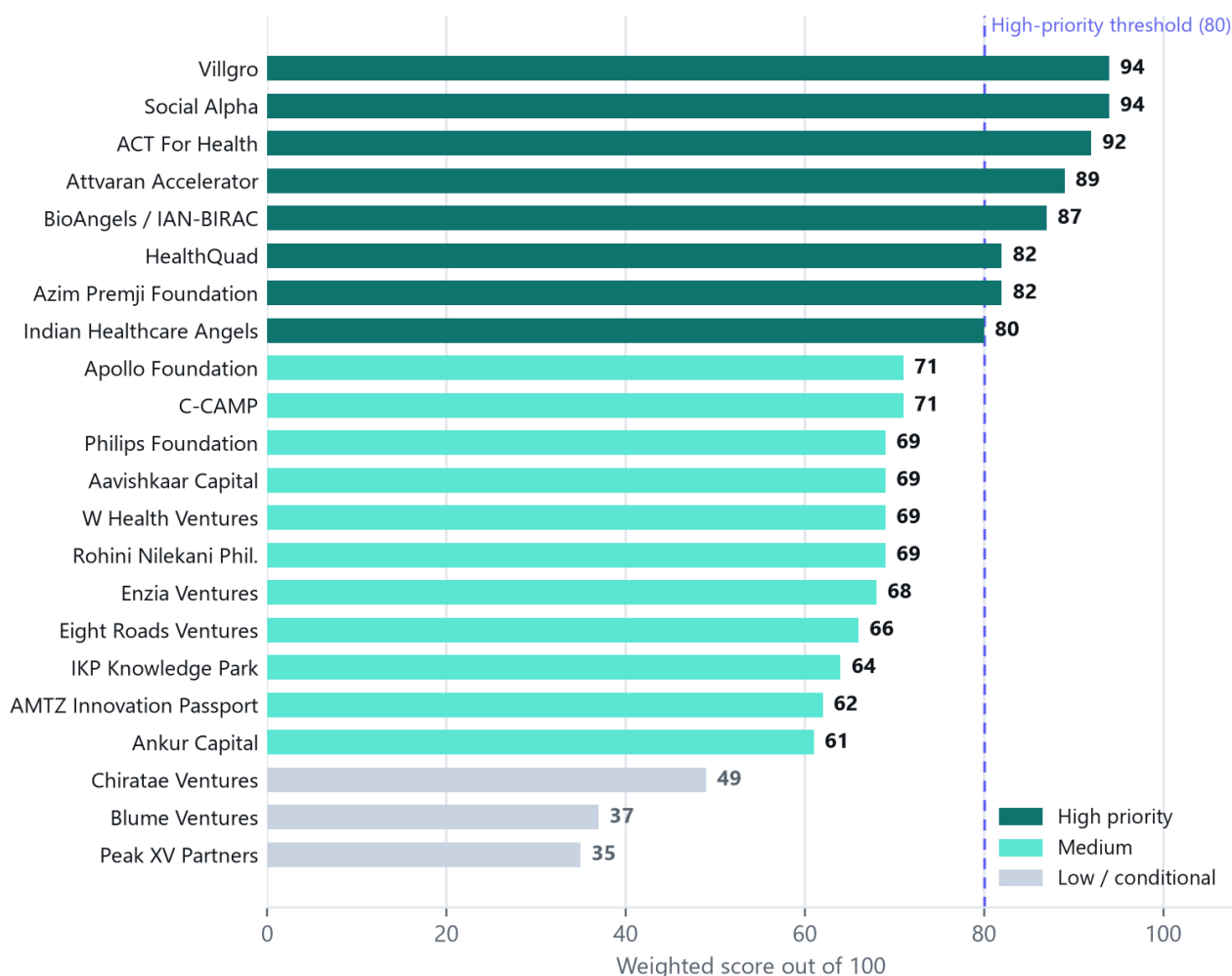


Figure 1 — Weighted scores. Social Alpha and Villgro tie at 94; Peak XV Partners scores 35.

THE HEADLINE FINDING

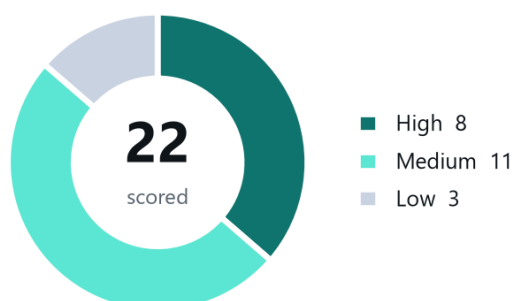
Peak XV, Blume and Chiratae — the three largest, best-known names on the longlist — finished 20th, 21st and 22nd.

Not because they lack money or healthcare interest, but because equity capital structurally cannot fund a Section 8 non-profit, and none showed evidence of backing one. The organisations that scored highest were accelerators, venture philanthropists and grant-making foundations whose capital is designed for exactly the kind of work aidKRIYA does.

Where the scores came from.

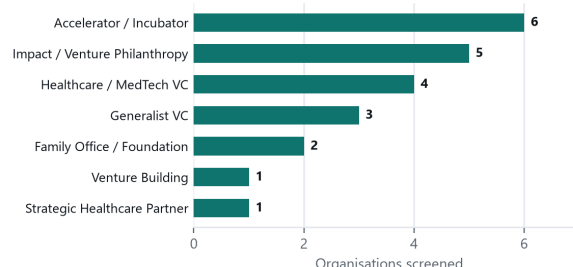
Priority distribution

Eight organisations cleared the 80-point threshold



Screened set by funder category

Accelerators and incubators dominate the shortlist



Figures 2 & 3 — Distribution of the 22 screened organisations by priority band and by funder type.

Criterion profile — top 10 shortlist

The same total can hide very different profiles



Figure 4 — Underlying 1–5 scores. Azim Premji Foundation and HealthQuad both total 82, but for opposite reasons.

The Ease-of-Access column is the story

Azim Premji Foundation and HealthQuad both score a perfect 5 on Strategic Fit and 5 on Funding Potential — and both drop to 2 on Ease of Access. Large, well-resourced funders are the hardest to reach. This is precisely why Ease of Access was capped at 10%: it should shape sequencing and the need for a warm introduction, not eliminate a strong fit.

Indian Healthcare Angels — the inverse case

IHA is the only shortlisted organisation scoring 5 on Ease of Access, with a published application route and a direct contact address. Its Funding Potential is the lowest in the top ten at 3. It earns its place as the **lowest-cost first conversation** on the list — useful for testing the pitch before spending a warm introduction on a harder target.

Ten organisations, ranked and reasoned.

Each entry carries a category, the aidKRIYA initiative it maps to, a recommended pitch angle and a named contact route. Scores are reproduced from the five criteria in Section 02.

#	ORGANISATION	CATEGORY	RELEVANT INITIATIVE	RECOMMENDED PITCH ANGLE	SCORE	PRIORITY
1	Social Alpha	Accelerator	aidABLE, Walker App, AIDothon	Assistive health innovation platform — mobility, digital walking assistance, chronic-care access	94	HIGH
2	Villgro	Accelerator	aidABLE, Walker App, Purplepages	Healthcare social enterprise moving from charity-led to repeatable, sustainable models	94	HIGH
3	ACT For Health / ACT Grants	Impact / Venture phil.	Walker App, Purplepages, peer matching, aidABLE	Digital/medtech chronic-care platform improving access to mobility, diagnosis and navigation	92	HIGH
4	Attvaran Accelerator	Accelerator	aidABLE, Walker App	Assistive-mobility platform improving independence, safety and dignity	89	HIGH
5	BioAngels / IAN-BIRAC	Healthcare VC	AIDothon, Walker App, aidABLE, diagnostics	Relationship and mentoring first; equity only via a future for-profit spin-out	87	HIGH
6	Azim Premji Foundation	Philanthropic foundation	Purplepages, shuKRIYA, community health	Disability prevention and assisted living — not "niche autoimmune disease"	82	HIGH
7	HealthQuad / HealthKois	Healthcare VC	Walker App, aidABLE, digital health	Network and ecosystem access, not a funding pitch	82	HIGH
8	Indian Healthcare Angels	Healthcare VC	Walker App, aidABLE, digital health	Investable healthcare-access platform with measurable patient outcomes	80	HIGH
9	C-CAMP	Accelerator	AIDothon, diagnostics, digital health, aidABLE	Lab-to-market support for RA/IBD digital health and diagnostics	71	MEDIUM
10	Apollo Foundation / Hospitals	Strategic partner	Diagnostics, awareness, patient education	Ecosystem partner extending Apollo outreach via screening and referral	71	MEDIUM

Table 4 — Recommended shortlist. Full 22-organisation register with criterion-level scores appears in the Appendix.

What every entry carries

- **Basic profile** — structure, founder, legal form, scale
- **Areas of interest** — stated thesis, not inferred
- **Past funding evidence** — named investees with sources
- **Fit rationale** — mapped to a specific aidKRIYA asset
- **Pitch angle** — the framing to lead with
- **Support type, contact route and risk**

A DELIBERATE CONTRADICTION, FLAGGED NOT HIDDEN

BioAngels scores 87 and lands in the High band — yet the written recommendation downgrades it to **Low / conditional**. The scoring model measures thematic and sector fit, and on that basis BioAngels is genuinely strong. What the model does not capture is that an equity angel platform structurally cannot invest in a Section 8 non-profit. Rather than quietly adjusting the score, the divergence was flagged to the client with the reason — the model stays honest, and so does the advice.

Three funders, **read closely.**

The workstream I owned. In each case the useful finding was not whether the funder liked healthcare — it was which door inside the organisation actually opens.

Azim Premji Foundation

82

Philanthropic foundation · India-only · not a VC, CSR arm or incubator

Strongest fit on the list

The critical finding — the right door is Disability, not Health

APF's *Health* vertical funds public-health-system strengthening in remote geographies: a weak fit. Its *Disability* vertical funds 70+ partner organisations doing therapeutic intervention, community-based rehabilitation, assisted living, caregiver support and — in their own words — "prevention of disability."

That maps almost one-to-one: early identification of RA/IBD to prevent irreversible joint damage *is* disability prevention; aidABLE *is* assisted living; shuKRIYAide *is* caregiver support. aidKRIYA is not adjacent to what they fund — it is what they fund, applied to a condition they have not yet covered.

Why the structure fits where it didn't elsewhere

Multi-year grants, not equity. No Section 8 barrier, no for-profit spin-out required, no exit expectation, no payer-market objection. Their stated philosophy — that "specific needs of the most vulnerable are worth serving in themselves" — inverts the objection every commercial funder raised. The niche scale that counted against aidKRIYA everywhere else is, here, the entire point.

TWO HONEST FRICTIONS

Geography: West Bengal is not among APF's stated priority states — a soft barrier, since their 1,000+ partners are nationwide. **Visibility:** application volume is high, so a warm introduction matters more than fit, which is already strong.

Scale	Endowment ~US\$29bn / ₹2.4 lakh crore, including 66% of Wipro's economic ownership — the largest private philanthropic endowment in India
Grant range	"A few lakh rupees to a few tens of crores"; tenures 1–10 years, mostly 3–4; 1,000+ NPO partners; Disability vertical: 70+ organisations
Precedent	₹500 crore to CMC Vellore; COVID commitments exceeding ₹2,000 crore; 1,100 rural creches; multi-year grants to organisations supporting persons with disability
Route	Grant application portal · philanthropy.registrations@azimpremjifoundation.org

BioAngels / IAN-BIRAC

87

Public-private angel platform · BIRAC (Govt of India) × Indian Angel Network · launched May 2022

Model score · advised Low

The thematic door is demonstrably open

Their portfolio already contains both an autoimmune therapeutics company (**Ahamune Biosciences**, vitiligo, \$5M Series A) and an assistive-mobility device company (**Arcatron Mobility**, commode wheelchairs, \$23.3M raised) — the two exact categories aidKRIYA operates in. Their own LinkedIn explicitly solicits assistive technology, naming "mobility devices such as wheelchairs and walkers."

Investment range ₹25 lakh–₹10 crore, syndicated across HNIs, angels, family offices and corporates. Launch target: ₹350 crore, 100+ startups in three years.

The recommendation: connection first, capital later

Equity angel investment cannot fund a Section 8 non-profit. But BioAngels describes what it offers as "**money, mentoring and critical sectoral expertise**" — and two of those three are available today without the equity question arising.

The relationship is genuinely two-way. aidKRIYA brings an engaged autoimmune patient community and real-world understanding of a group companies in this space struggle to reach. Approach as a mentoring and network channel; let any funding conversation develop from a spin-out later.

When the right answer is "don't ask for money."

82

HealthQuad / HealthKois

Early-growth healthcare VC · venture arm of Quadria Capital (US\$4bn+ AUM) · Fund III
target ₹1,700 Cr

Low as funder · Medium as network

Why not as a funder

- Equity model cannot fund a Section 8 company.
- Cheque size US\$1–5M (₹8–42 Cr) — far beyond aidKRIYA's stage.
- Thesis explicitly seeks "bold, scalable, category-defining companies" — the opposite of aidKRIYA's profile.

Note: HealthQuad and HealthKois separated in 2025 (Quadria took full ownership of HealthQuad). This assessment covers HealthQuad only.

Why it still earns a place

HealthQuad's own stated edge is access to Quadria's network of hospitals, pharma, insurers, diagnostics and policymakers, used to enable validation, adoption and access. That network — plus portfolio companies like **ekincare** (corporate wellness), **Redcliffe Labs** (diagnostics) and **GoApptiv** (rural last-mile reach) — is exactly what aidKRIYA needs and cannot build alone.

The warm, mission-led entry point is co-founder **Dr Amit Varma**, a critical-care physician who describes himself as passionate about advancing healthcare through academia, research and non-profit endeavours — a story-and-mission audience, not a pure-returns one.

THE CATCH, STATED PLAINLY

HealthQuad states that ESG and social impact sit "at the heart" of their philosophy — but their own wording carries the condition: they fund companies that "not only deliver strong financial returns but also create meaningful social impact." Impact is always *and*, never *instead*. The ESG hook gets aidKRIYA into the room; it cannot carry an investment alone.

The pattern across all three

Each of these funders has a genuine thematic overlap with aidKRIYA, and each requires a different *door*. APF's door is a vertical (Disability, not Health). BioAngels' door is mentoring, not capital. HealthQuad's door is its portfolio network, not its fund. In every case the research changed what the client should **ask for**, not just who to ask.

What this changed for the client

- **Reframed the ask** — from "investment" to route-specific: grant, incubation, mentoring or network access.
- **Reordered outreach** — non-VC routes first; investor routes deferred until evidence exists.
- **Named the evidence gaps** — user feedback, a signed healthcare or NGO partner, and a clear revenue structure.
- **Separated the pipeline** — priority routes vs conditional routes, so effort follows probability.

Funders want to see revenue, not just need.

Every route in Section 03 shares one expectation: seed capital should build sustainable income, not subsidise indefinitely. Five revenue models were built and stress-tested against a single question — would the buyer actually pay, and can aidKRIYA actually deliver it?

Modelled revenue by stream

Gross annual revenue against net contribution after setup and running costs

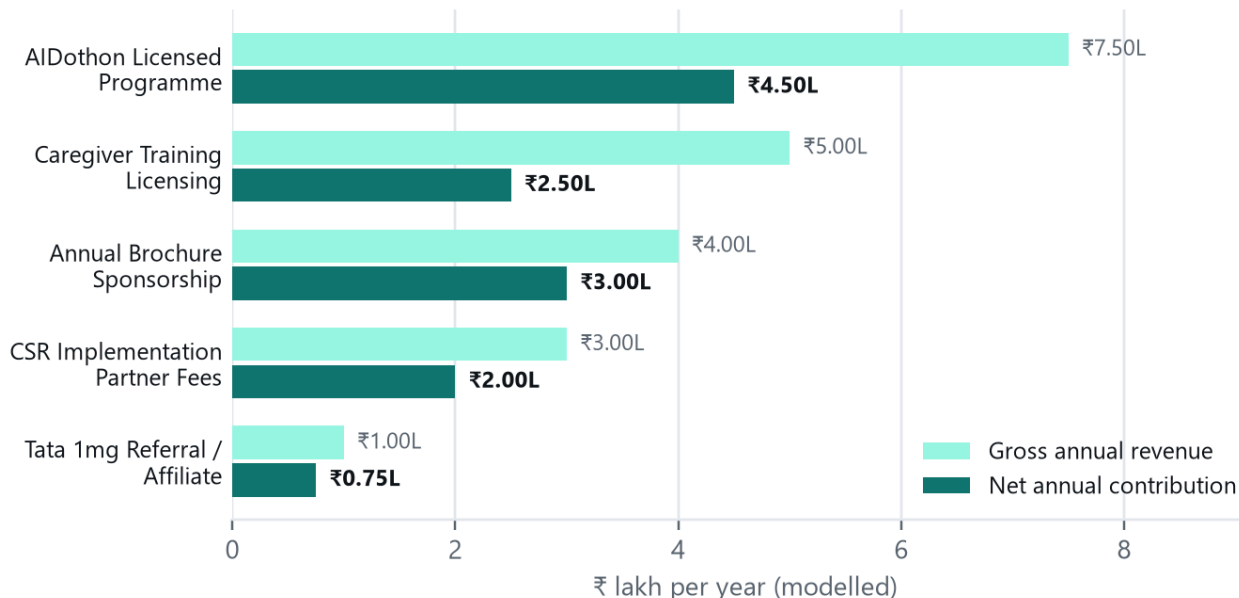


Figure 5 — Illustrative modelling. Combined gross ₹20.5 lakh; combined net contribution ₹12.75 lakh per year at modelled volumes.

REVENUE STREAM	WHO ACTUALLY PAYS	PRICE (₹)	VOL/YR	NET P.A. (₹)	PRIORITY	KEY DEPENDENCY
Tata 1mg referral / affiliate	Tata 1mg platform	500	2,000	75,000	HIGH	Formal revenue-sharing agreement
AIDothon licensed programme	Corporate sponsors (revised from colleges)	15,000	50	4,50,000	HIGH	Packaged curriculum and partner deck
Annual brochure sponsorship	Corporate CSR partners	50,000	8	3,00,000	MEDIUM	Reframe as CSR documentation package
CSR implementation partner fees	Corporate donors, foundations	30,000	10	2,00,000	MEDIUM	CSR-1 registration — the single gate
Caregiver training & placement	Employers, families, CSR sponsors	3,000–5,000	—	2,50,000	MEDIUM	Clinical endorsement of the certificate

Table 5 — Revenue models with modelled economics. No model charges patients or caregivers ("Warriors") directly.

THE GOVERNING PRINCIPLE

Warriors should never bear the financial burden. Every model is structured so that corporates, CSR sponsors, institutions or patrons pay, while patients and caregivers continue receiving support free or subsidised. Any paid element — sponsored listings, pharma partnerships, referral arrangements — must be optional or third-party funded and handled transparently. Revenue generation must protect the mission, not compete with it.

Two models that needed rebuilding.

Both original proposals asked the wrong party to pay. Testing them against real comparables showed the buyer was elsewhere in the chain.

Caregiver training — from a ₹1 lakh licence to a three-layer engine

The original model licensed a caregiver curriculum to home-healthcare companies at ₹1 lakh. Three problems: the curriculum does not exist as a product, the buyer pool is perhaps 10–15 companies nationally, and no employer trusts an uncredentialed NGO's training at that price. Restructured, the same assets produce revenue without ever charging the person who cannot pay.

Layer 1 — Free training

The pipeline. Open to anyone.

Basic chronic-disease caregiving: mobility support for joint stiffness, IBD daily routines, medication awareness, flare warning signs. Trainees are then placed with underprivileged Warrior families free or subsidised.

Mission served: the family gets care it could never afford; the caregiver gets the clinical exposure that makes everything after it credible.

Layer 2 — Paid certification

The upgrade. ₹3,000–₹5,000 for assessed modules — joint-stiffness management, flare recognition, medication scheduling, mobility-aid handling, burnout prevention — plus practical evaluation.

Low enough that someone serious about employment can afford it (or a CSR sponsor covers it), high enough to filter for commitment. Backed by real Layer 1 practice, not classroom theory.

Layer 3 — Placement

The revenue engine. Certified caregivers enter a placement marketplace: families who can pay, home-healthcare companies, corporate wellness programmes, hospitals needing post-discharge care.

aidKRIYA takes a placement fee (₹5,000–₹15,000, or a monthly percentage). The hirer was going to pay for a caregiver anyway — now they get a disease-specific one.

THE CYCLE

Free training → placed with a poor family for experience (mission) → paid certification (revenue) → placed with a paying family or corporate (revenue) → placement fees fund the next round of free training. **The mission funds the revenue and the revenue funds the mission.** One gate determines whether it works: the certificate must carry trust. A clinical endorsement — ideally the Indian Rheumatology Association co-branding it — closes that gap in a single move.

AIDothon — charge the corporate, not the college

The original model licensed AIDothon to colleges at ₹15,000. But free health hackathons are everywhere: Smart India Hackathon alone reaches 5 million students at zero cost with ₹1.5 lakh prizes and government branding. No college pays ₹15,000 for a disease-specific hackathon when superior alternatives are free.

How the restructured model works

A corporate sponsors an AIDothon cycle and submits a **real problem statement** their business needs solved. Student teams build working prototypes over several weeks, guided by aidKRIYA's disease expertise and access to its patient community, with academic backing from IIT Ropar.

The corporate gets first access to winning solutions and teams, an 80G receipt, brand presence and a CSR impact story — curated R&D, a talent pipeline and CSR credit from one sponsorship. Colleges and students participate free, the only price they will accept.

The ask is not new money — it is the same money these corporates already give, redirected into a structure that returns more value to them.

Why it is viable now

- **Relationships are live** — eight named corporate pillars plus Tata 1mg and ImpactGuru already give money.
- **Academic partner is proven** — IIT Ropar across four AIDothon editions.
- **The outcome is documented** — Flexirelief SmartCare, founded by an AIDothon alumnus, now incubated at IIT Patna with government grants and a 10-person team building RA care solutions.
- **Marginal cost is low** — one sponsor at ₹2–5 lakh covers operating cost and nets revenue from day one.

What the client should do next.

Sequenced by readiness rather than ambition. Nothing in the first phase requires new infrastructure or new relationships.

Phased plan

NOW — BEFORE ANY OUTREACH

Build one funder-facing package

A short pitch deck, a one-page organisation profile, a clear funding ask, product status, pilot or user evidence, partner interest, and a simple explanation of patient impact. Confirm the amount actually required. This is a packaging exercise, not a creation exercise — most of the content already exists across four annual brochures.

NEXT 3 MONTHS

Launch what needs no new infrastructure

aidABLE sponsored mobility packages (the Sahayatha and Creativiti Council supply chains already work), corporate health-awareness workshops, and a restructured patron giving circle at ₹500–₹1,000/month. In parallel, open the accelerator and grant routes: Social Alpha, Villgro, ACT For Health and Attvaran.

3 – 9 MONTHS

Formalise and register

Complete **CSR-1 registration** — without it aidKRIYA cannot appear on the MCA's approved implementing-agency list, which is where corporate CSR teams begin searching. Convert the Tata 1mg collaboration into an affiliate agreement, pilot the corporate-sponsored AIDothon cycle, and resolve IP ownership on the caregiver curriculum.

9 – 18 MONTHS

Only then, the investor routes

Walker App premium features, Purplepages sponsored listings and B2B licensing to hospitals or pharma require digital traction and product maturity that does not exist yet. Track them as opportunities; do not resource them until earlier models generate stable income.

THE CORE RECOMMENDATION

Do not lead with a venture-capital pitch.

Position aidKRIYA as a **health-impact platform for chronic-condition patients** and open four routes in order: accelerators and incubators, health-impact and grant-style funders, community-health foundations — and healthcare investors only after stronger traction exists.

Materials to prepare

- **Impact one-pager** — reach in hard numbers, consolidated from four brochures
- **CSR sponsorship catalogue** — menu format with price tags and defined returns
- **AIDothon programme deck** — 10–15 slides, outcomes and the Flexirelief story
- **Audited financials summary** — framed as responsible stewardship, not losses
- **Caregiver curriculum document** — modules, objectives, assessment, certification

Deepen before widening

Existing partnerships — Tata 1mg, ImpactGuru, Sahayatha, Rossari Biotech, Creativiti Council — are aidKRIYA's strongest credibility asset. Formalise them first, then ask each existing partner for **one introduction per quarter**. A warm introduction outperforms a cold approach at every funder on this list.

Expected outcomes of the engagement

A clearer pipeline

Priority and conditional routes separated, so outreach effort follows probability rather than brand recognition.

A defensible direction

Non-VC routes prioritised ahead of pure venture capital, with the reasoning documented and reproducible.

A decision basis

First approaches, postponed investor routes and named evidence gaps — a plan the client can execute without us.

Full scored register.

All 22 screened organisations with criterion-level scores. Weighted total = (SF×0.35 + LS×0.25 + FP×0.20 + EA×0.10 + UT×0.10) × 20.

ORGANISATION	CATEGORY	SF 35%	LS 25%	FP 20%	EA 10%	UT 10%	WEIGHTED	PRIORITY
Social Alpha	Accelerator	5	5	4	4	5	94	HIGH
Villgro	Accelerator	5	5	4	4	5	94	HIGH
ACT For Health / ACT Grants	Impact / Venture phil.	5	5	4	3	5	92	HIGH
Attvaran Accelerator	Accelerator	5	4	4	4	5	89	HIGH
BioAngels / IAN-BIRAC	Healthcare VC	5	4	4	3	5	87	HIGH
Azim Premji Foundation	Family office	5	3	5	2	4	82	HIGH
HealthQuad / HealthKois	Healthcare VC	5	3	5	2	4	82	HIGH
Indian Healthcare Angels	Healthcare VC	4	4	3	5	5	80	HIGH
C-CAMP	Accelerator	4	3	3	4	4	71	MEDIUM
Apollo Foundation	Strategic partner	4	3	4	3	3	71	MEDIUM
Rohini Nilekani Phil.	Family office	4	3	4	2	3	69	MEDIUM
W Health Ventures	Venture building	4	3	4	2	3	69	MEDIUM
Aavishkaar Capital	Impact / Venture phil.	4	3	4	2	3	69	MEDIUM
Philips Foundation Impact	Impact / Venture phil.	5	2	4	1	3	69	MEDIUM
Enzia Ventures	Impact / Venture phil.	3	3	3	5	5	68	MEDIUM
Eight Roads Ventures	Healthcare VC	4	2	5	2	2	66	MEDIUM
IKP Knowledge Park	Accelerator	3	3	4	3	3	64	MEDIUM
AMTZ Innovation Passport	Accelerator	3	3	3	4	3	62	MEDIUM
Ankur Capital	Impact / Venture phil.	2	3	4	4	4	61	MEDIUM
Chiratae Ventures	Generalist VC	3	2	2	3	2	49	LOW
Blume Ventures	Generalist VC	2	1	2	3	2	37	LOW
Peak XV Partners	Generalist VC	2	1	2	3	1	35	LOW

Table 6 — Full register. SF Strategic Fit · LS Likelihood of Success · FP Funding Potential · EA Ease of Access · UT Urgency / Timing.

Sources — funder websites, annual reports and portfolios; Crunchbase; Tracxn; LinkedIn; BIRAC and AT2030 programme pages; Bain & Company *India Philanthropy Report 2025*; CSRBOX *Indian CSR Outlook Report 2025*; EY; Bridgespan × NATHEALTH; aidKRIYA brochures and briefing materials.

Limitations — scores reflect public information as at July 2026 and the client's stated priorities. Rows where deal-level amounts are undisclosed are flagged as needing primary confirmation. The framework ranks thematic and practical fit; legal-structure constraints are handled separately.

Verified completion.

Issued by King's College London and Practera on completion of the Finance PGT micro internship — the two-week industry challenge that produced the work in this report.



Programme	King's Micro Internship: Finance PGT — a 2-week authentic industry challenge
Awarded to	Dhruv Kamdar
Issued	July 2026 · London, UK
Signatories	Beau Leese, CEO of Practera · Markus Stretz, Manager, Work-Related Learning, King's College London

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Dhruv Kamdar

Data & Finance Analytics · London, UK

WHAT I TOOK FROM IT

Clean data first. Then the opinion.

The value in this engagement was not finding funders — anyone can list VC firms. It was building one consistent, reproducible scale, applying it to every candidate without exception, and then being willing to say that the highest-scoring name on the sheet was still the wrong answer for structural reasons the model could not see. Reconciling contradictory public sources, documenting every score back to its inputs, and flagging the divergences rather than smoothing them is the same discipline I bring to data quality and governance work.

Reconciliation

Funder data from portfolios, filings, news and databases that routinely disagreed — resolved to a single scored master sheet.

Reproducibility

Every one of the 22 weighted scores reproduces exactly from its five documented inputs and a published rubric.

Communication

Findings translated into a sequenced action plan and presented to the client's Managing Director.

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